

Optimizing Specialty Medication Access Through Multi-Step Order Transmittal (MSOT) Workflows: A Single Health System Evaluation

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BACKGROUND

- Specialty pharmacy workflows face growing challenges from payer lockouts, Limited Distribution Drug (LDD) restrictions, and complex access pathways.¹
- Multi-Step Order Transmittal (MSOT) within an electronic health record (EHR) centralizes and streamlines prescription access across ambulatory and specialty clinics.¹
- Unlike conventional provider initiated prior authorization processes, MSOT automatically routes complex prescriptions via embedded EHR logic to a centralized team, ensuring earlier and more coordinated access support without additional provider burden.¹
- This approach aims to improve prescription capture rates, reduce turnaround time, and increase visibility into access barriers.

OBJECTIVES

To evaluate the impact of MSOT implementation across Neurology, Rheumatology, and Cardiology clinics on:

- Prescription turnaround time (TAT)
- Prescription capture rate
- LDD and payer lockout identification
- Provider satisfaction via Net Promoter Score (NPS)

METHODS

- Retrospective, pre/post descriptive cohort analysis conducted from March 2024 – April 2025 at a single academic medical center.
- Evaluated outcomes before and after centralized pharmacy intervention through MSOT implementation.
- Descriptive analysis only.

Key interventions: Benefits Investigation (BI), Prior Authorization (PA) completion, and prescription fulfillment.

Data sources: Epic, MSOT queue, pharmacy dispense records, and Qualtrics survey.

Limitations: Manual baseline tracking, incomplete PA capture, lower MSOT volume during phased rollout.

Pre-MSOT data (March 2024 – September 2024):

- TAT from intervention need to dispense (CoverMyMeds (CMM) and Epic).
- LDD and payer lockout tracked manually in Excel.
- Capture rate: % of prescriptions filled by UVA.

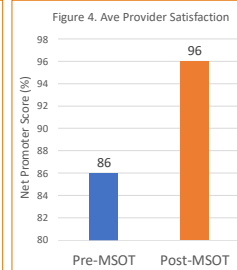
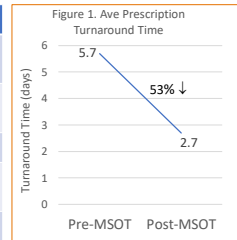
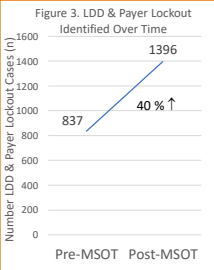
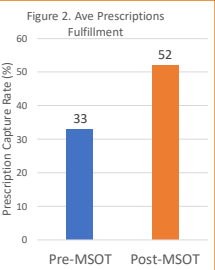
Post-MSOT data (October 2024 – April 2025):

- TAT from MSOT entry to adjudication or intervention through dispense.
- LDD and payer lockout from MSOT order questionnaires.
- Capture rate: % of MSOT prescriptions filled by UVA.
- NPS: % change in provider satisfaction from baseline.

RESULTS

Table 1. MSOT Performance Metrics

Metric	Pre-MSOT	Post-MSOT
Prescription Reviewed, n	8,132	7,316
Turnaround Time, days	5.7	2.7
Capture Rate, % (n)	33% (2683)	52% (3804)
LDD & Payer Lockout Cases, n	837	1,396
Provider NPS, % (n)	86% (13)	96% (15)



DISCUSSION

- The observed 53% reduction in turnaround time not only streamlines provider workflows but supports earlier therapy initiation which is critical in managing chronic and progressive conditions.
- Higher prescription capture within the health system also ensures closer care coordination and potential cost savings through avoidance of fragmented dispensing.
- The significant increase in prescription capture rate from 33% to 52% demonstrates better retention of prescriptions within the health system.
- LDD and payer lockout identification increased by 40%, enabling earlier access intervention.
- Provider NPS improved from +10% (96%) post-MSOT, reflecting enhanced trust and satisfaction with pharmacy services amid reduced administrative burden.
- Manual tracking and phased rollout limited data completeness, highlighting opportunities for broader automation.

CONCLUSIONS

- MSOT offers scalable and meaningful benefits for health system specialty pharmacy programs.
- At UVA Health, MSOT implementation led to faster patient access, higher prescription capture, and proved provider satisfaction.
- The workflow also enabled strategic insight into access barriers such as payer and manufacturer restrictions.
- Expanding MSOT beyond pilot clinics could extend these benefits across more therapeutic areas.
- Integrating real-time analytics may further enhance performance and uncover additional value in complex medication pathways.¹⁻³

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